

Anterior uveitis phenotypes that change the workup

Part 2 | A phenotype is a test-selection tool

Describe the inflammation before ordering an undirected panel.

Axis	Why it changes reasoning
Acute recurrent versus chronic	Different associations and cumulative burden
Unilateral alternating versus bilateral	Different systemic and infectious possibilities
Granulomatous signs and iris change	Targeted phenotype-dependent investigations
Pressure and posterior involvement	Look beyond the anterior cell count

SUN classification criteria support research classification; they are not a replacement for clinical diagnosis or an exclusion rule for every atypical patient. Painful recurrent acute unilateral or alternating attacks with fibrin support an HLA-B27-associated phenotype. History should specifically explore inflammatory back symptoms, bowel disease and other relevant systemic features.

TINU can outlast the renal symptoms

A quiet systemic history or normal creatinine does not exclude tubular disease.

Renal and ocular courses may be offset

Earlier systemic symptoms

Renal injury



Uveitis



Later or recurrent ocular activity

Original conceptual timeline: renal and ocular manifestations need not be synchronous.

Bilateral anterior uveitis after a systemic illness warrants a renal history and appropriate urinalysis or tubular assessment when the phenotype fits. Urinary beta-2 microglobulin can support recognition of tubular injury, but it is not uniquely specific for TINU. Coordinate renal evaluation and exclude competing causes before naming a combined syndrome.

Fuchs cells are not a routine steroid target

Treat visual and pressure complications without indefinitely chasing trace cells.

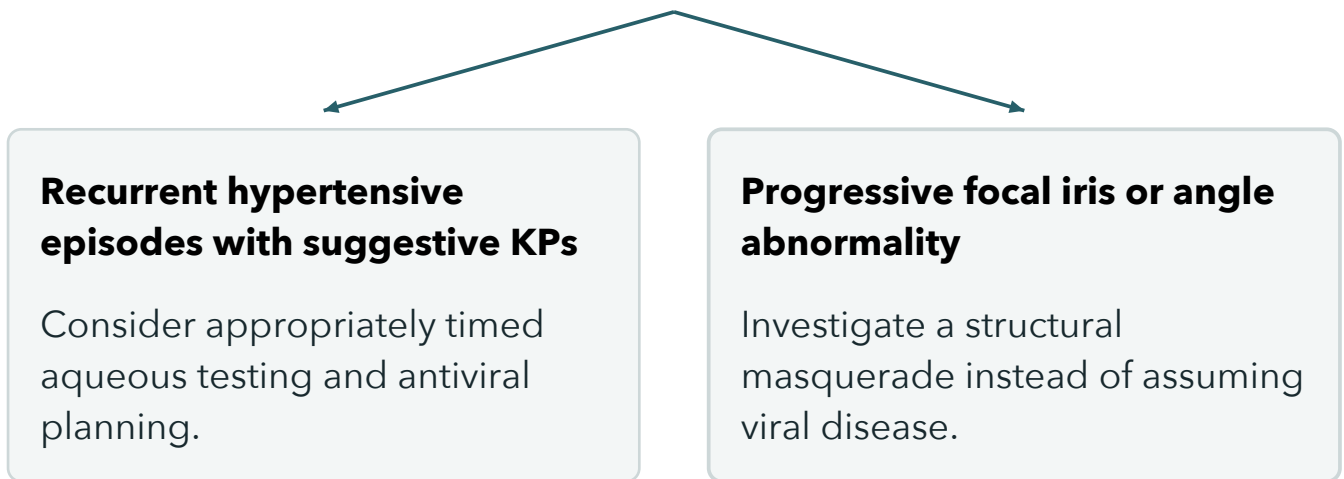
Typical feature	Practical implication
Diffuse stellate KPs	May accompany low-grade chronic inflammation
Iris stromal atrophy	Heterochromia may be subtle or absent
Absent posterior synechiae	Supports the phenotype, not definitive alone
Cataract or pressure damage	Often drives treatment decisions

Repeated steroid can produce harm without clearing the characteristic low-grade findings. Confirm that the presentation fits and investigate discordant features. Vitreous cells may occur. Cataract and glaucoma require their own assessment; an unexpected focal iris mass or progressive angle lesion should not be explained away by a Fuchs label.

Hypertensive viral uveitis needs directed evidence

Pressure disproportionate to inflammation should trigger reconsideration.

Does the phenotype support viral anterior disease?



CMV can cause recurrent or chronic anterior uveitis and endothelial injury. Sampling timing and assay limitations matter; a negative result during low activity may not settle a compelling case. Drug route, duration and relapse surveillance depend on the phenotype and patient factors. Pressure control does not eradicate infection.

A treatment response is not a diagnostic proof

Infection, drug exposure and tumours can all produce inflammation.

Discordant cue	Next reasoning step
Temporal relationship to a new drug	Review causality and alternatives
Progressive focal pupil or iris distortion	Image the iris and ciliary body
Sentinel vessel or adjacent angle lesion	Seek ocular-oncology assessment
Partial steroid response	Secondary inflammation can be suppressed

Before systemic escalation, revisit unilateral structural change and the complete medication timeline. UBM characterizes hidden anterior uveal anatomy and helps guide further assessment. Do not treat an autoimmune laboratory result as proof that a focal lesion is benign, or use prolonged steroid response as a substitute for imaging.